1. Stakeholder Identification & Priorities

To ensure the project is responsible and community-responsive, we focus on two primary stakeholder groups: **End-Users / Community Members** and **Mental Health Professionals / Clinical Experts**.

Stakeholder Group 1: End-Users and Community Members

Who They Are: Individuals navigating high-pressure daily environments, social media forums (such as Reddit or Dreaddit), and academic or professional settings who experience chronic stress.
Why They Matter: They are the primary subjects and direct users whose text data and emotional well-being are directly affected by model classifications.
Key Priorities: Data privacy, dignity, freedom from unwarranted algorithmic judgment, and complete transparency regarding how their personal expressions are processed and analyzed.

Stakeholder Group 2: Mental Health Professionals and Clinical Experts

Who They Are: Licensed counselors, psychologists, and clinical researchers specializing in stress management and burnout prevention.
Why They Matter: They provide essential domain expertise to validate whether definitions of "stress distress" align with clinical realities rather than arbitrary metrics.
Key Priorities: Clinical accuracy, protection against misdiagnosis, avoidance of harmful stigmas, and ensuring automated tools support rather than replace genuine human care.

2. Potential Benefits and Risks

For End-Users and Community Members

Benefits: Provides early awareness of personal stress trends, empowering individuals to manage mental health proactively before reaching severe burnout.
Risks & Harms: Potential data breaches exposing sensitive emotional states, heightened anxiety caused by false-positive flags, and reduced autonomy if individuals feel constantly monitored by automated systems.
Social & Economic Impact: Risk of social stigma if sensitive mental health classifications inadvertently leak into professional or academic records.

For Mental Health Professionals

Benefits: Offers valuable population-level insights on stress trends, helping direct public health resources and interventions more effectively.
Risks & Harms: Automation bias where users overly rely on a language model over professional clinical advice, potentially delaying or bypassing necessary medical intervention.
Ethical & Cultural Considerations: Variations in how stress and emotional distress are expressed across cultures, which models trained primarily on Western-centric datasets might misinterpret.

3. Engagement Method: The World Café

To engage stakeholders collaboratively, we select the **World Café** method as outlined in the Action Catalogue framework. This approach is highly effective for collaborative learning, generating new ideas, and enabling joint decision-making on complex strategic issues by integrating technical knowledge with collective human wisdom.

The method is particularly suitable for these stakeholder groups because it breaks down power dynamics between technical developers, clinicians, and everyday users. Hosted in an informal, hospitable setting with round tables, it encourages equal contribution and ensures vulnerable end-user perspectives are meaningfully included.

Conducting the Engagement

1. Setup: Tables are arranged informally with small groups of 4 to 5 participants mixing developers, end-users, and clinicians together.
2. The Rounds: Participants engage in three 20-minute rounds of structured conversation centered around core project questions (e.g., "How can we display stress scores without inducing panic?").
3. Cross-Pollination: After each round, a table host stays behind to brief incoming members while other participants rotate to new tables to build upon earlier ideas.
4. Harvesting: Key insights are visually synthesized through real-time graphic recording to directly inform technical and design decisions.

4. Influence on Project Design

Feedback gathered from the stakeholder engagement sessions will directly shape the technical pipeline across four key areas:
Dataset Adjustments: Insights from diverse end-users will guide audits of training data for demographic and cultural blind spots, ensuring regex cleaning and text filters do not penalize unique slang or regional expressions of stress.

Model Design: Clinical feedback regarding false-positive anxiety will inform adjustments to classification thresholds, moving from binary cutoffs to graded confidence scores that require higher certainty before flagging distress.
Outputs and UI Design: Alarming labels like "High Distress Detected" will be replaced with supportive, empathetic phrasing paired with direct access to professional mental health resources.

- **Deployment Decisions:** Privacy concerns will enforce key deployment constraints, guaranteeing that all text processing remains transient and ephemeral with zero long-term retention of personal communications.